

SEP Compliance Cheat Sheet

Print this. Keep it on your screen. Use it before every SEP enrollment. This is your pre-flight checklist.

The difference between a clean submission and a compliance review is five minutes of verification. This guide covers what to check, what to avoid, and what happens when you get it wrong.

Before Every SEP Enrollment

Ask yourself these five questions before submitting. If you cannot answer all five with confidence, stop and verify.

#	QUESTION	WHY IT MATTERS
1	What is the qualifying event?	Can you name the specific life event? (Moved, lost coverage, gained Medicaid, chronic condition, etc.)
2	When did it happen?	Do you have the date? Is the SEP window still open?
3	Does the beneficiary understand?	Can they explain in their own words why they qualify? "Okay" is not verification.
4	Is this the right SEP code?	Is there a more beneficial enrollment period available per the CMS hierarchy?
5	Is the plan type valid for this SEP?	INT = D-SNP only. DEP = PDP only. ICEP = MA/MAPD only. CDC = MA-only. Check before submitting.

The 5 Most-Watched SEPs

These are the codes carrier compliance teams track most aggressively. Exceed the benchmark and expect an audit.

CODE	NAME	BENCHMARK	WHAT GETS FLAGGED
DST	Disaster SEP	~15%	Using it as a standalone SEP. Marketing or advertising it. No matching FEMA declaration.
MOV	Moving SEP	~10%	PO Box changes submitted as moves. No address verification with Social Security. MARx still shows old address.
LCC	Loss of Creditable Coverage	~4%	Using it when the beneficiary stopped paying premiums (voluntary loss). Must be involuntary.
ACC	Accessible Format	~3%	Using it as a convenience code. Beneficiary did not actually request accessible materials.
PAP	SPAP	~5%	Beneficiary not actively enrolled in a qualifying state pharmaceutical assistance program.

Red Flags That Get You Flagged

These patterns trigger automated compliance alerts.

- High DST volume without matching FEMA declarations in your service area
- MOV enrollments without corresponding address changes in MARx
- ACC usage above 3% of total enrollments
- Multiple different SEP codes used for the same beneficiary in a short period
- SEP enrollments consistently submitted on the last day of windows
- LCC for beneficiaries whose coverage lapsed due to non-payment
- INT enrollments into D-SNPs that are not FIDE, HIDE, or AIP
- Give-back plan enrollments for Medicaid beneficiaries

What Happens When You Submit the Wrong SEP

Five escalating consequences. Each stage is harder to recover from than the last.

STAGE	WHAT HAPPENS	IMPACT
1. Rejection	Enrollment denied at submission	Beneficiary has no plan. You wasted their time. The application is returned, and the beneficiary must start over with the correct SEP — if one still exists.
2. Retro disenrollment	Accepted enrollment later audited and reversed	Beneficiary loses coverage they thought was active — sometimes months later. Claims incurred during that period may become the beneficiary's responsibility. They lose trust in you.
3. Compliance review	Your enrollment patterns flagged by carrier	You receive a corrective action plan. Mandatory retraining is assigned. Your enrollment activity is monitored closely for 90+ days. Every future submission gets extra scrutiny.
4. Suspension	Enrollment privileges suspended	You cannot submit any enrollments — sometimes across all carriers, not just the one that flagged you. Your book of business is frozen. Downline agents may also be affected.
5. Termination	Carrier contract terminated for cause	Permanent carrier blacklist. Potential CMS sanction reported to other carriers. Other contracts may be terminated as a result. Rebuilding your career requires starting from scratch.

The DO NOT List

Bright-line rules. No exceptions. No gray area. Each rule includes the reason — know the why, not just the what.

DO NOT enroll Medicaid beneficiaries in give-back plans

Medicaid already pays their Part B premium — the give-back benefit has no value to them.

Use D-SNPs instead. Give-back enrollments for Medicaid beneficiaries are flagged as unsuitable recommendations and may result in retro disenrollment.

DO NOT advertise or market the Disaster SEP

CMS prohibits proactive marketing of DST. The beneficiary must raise the situation themselves.

You cannot send mailers, run ads, or proactively call about DST. If you are caught marketing it, the carrier will terminate your contract.

DO NOT assume or infer an SEP

Guessing that someone qualifies is not verification — it is a compliance violation.

The qualifying event must be confirmed by the beneficiary and verifiable in the system. Document the conversation.

DO NOT use ACC for beneficiaries who do not need accessible format materials

ACC is one of the most audited codes. Carrier benchmark is 3% — exceed it and you will be reviewed.

The beneficiary must have actually requested accessible materials (large print, Braille, audio, etc.). Convenience use triggers immediate pattern flags.

DO NOT submit an enrollment without verifying the election period with the beneficiary

CMS requires Election Period Verification as a documented conversation, not a checkbox.

The beneficiary must confirm and understand which enrollment period they are using and why. Skipping this step is a standalone compliance violation.

DO NOT use DST as a standalone SEP

DST extends a missed window — it does not create a new enrollment right.

The beneficiary must have had another enrollment period open during the disaster. No underlying SEP = no DST eligibility.

DO NOT use LCC for voluntary coverage loss

LCC requires involuntary loss. Non-payment of premiums is considered voluntary.

If the beneficiary missed premium payments and their coverage lapsed, LCC does not apply. This is one of the most common misuses carriers catch in audits.

DO NOT submit INT without confirming the D-SNP is integrated

INT requires FIDE, HIDE, or AIP designation. Not all D-SNPs qualify.

The beneficiary's Managed Care Organization (MCO) must also match. Submitting INT for a non-integrated D-SNP results in automatic rejection or retro disenrollment.

DO NOT use OEP for dual-eligible beneficiaries who have INT or DEP

INT and DEP are monthly and repeatable. OEP is one-and-done per year.

Using OEP when INT or DEP is available wastes the beneficiary's annual OEP. If they need to make a change later, they will have no remaining options.

DO NOT hold applications

CMS requires applications to be submitted within 24 hours of receipt.

You cannot hold them until a more convenient time, a better effective date, or a different enrollment window. This is a termination offense — grounds for immediate contract removal.

Quick Escalation Reference

When you are not sure, follow these four steps in order. Do not skip ahead.

1 Check The Certainty System at /sep-check

All 37 codes with eligibility criteria, plan type rules, and signal classification. Start here every time.

2 Check carrier portal

Humana (Vantage/Mentor DMS-024), UHC (Election Period Booklet), Devoted (Agent Portal SEP tracker). Carrier-specific rules override general guidance.

3 Call your upline

They have seen the edge cases. A 2-minute call can prevent a 2-month compliance review.

4 Contact carrier compliance

A pre-submission question is always better than a post-audit correction. Compliance teams would rather help you get it right than clean up after you get it wrong.

THE SINGLE BEST HABIT

If you are not 100% sure the SEP is valid, pause and verify. The five minutes it takes to confirm will save you weeks of dealing with a compliance review. Ask before you submit.

SEP Plan Type Quick Reference

Not every SEP allows enrollment in every plan type. Check before you submit.

SEP CODE	MA	MAPD	PDP	D-SNP	MA-ONLY
INT	—	—	—	Y	—
DEP	—	—	Y	—	—
MCD	—	Y	Y	—	—
NLS	—	Y	Y	—	—

ICEP	Y	Y	—	—	Y
IEP	—	Y	Y	—	—
OEP-I	Y	Y	—	—	—
LTC	—	—	Y	—	—
CDC	—	—	—	—	Y
INV	—	—	Y	—	—
OSD	—	—	Y	—	—
12G	—	—	Y	—	—
12J	—	—	Y	—	—
MOV	Y	Y	Y	—	—
LEC	Y	Y	Y	—	—
CSN	—	—	—	Y	—

Y = Eligible plan type — = Not eligible

Key Rules to Remember

- INT is D-SNP only. Full Medicaid required. MCO must match.
- DEP is PDP only. Any Medicaid or LIS level qualifies.
- ICEP is MA/MAPD only. Used when Part A and Part B effective dates differ.
- CDC is MA-only. Beneficiary must have other creditable drug coverage.
- CSN is D-SNP (C-SNP) only. Must enroll into a plan that matches the qualifying condition.
- MOV and LEC are the broadest — eligible for MA, MAPD, and PDP.

This cheat sheet is a quick-reference guide from Mega Care Insurance. For complete SEP eligibility criteria and real-time verification, use The Certainty System at [/sep-check](#). For carrier-specific guidance, consult carrier compliance portals.